

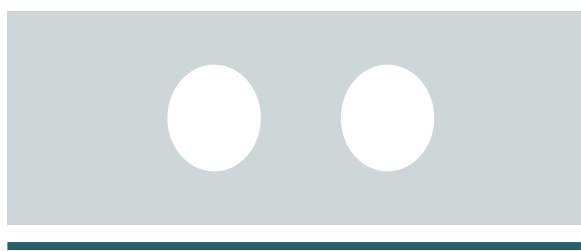
Macular OCT mimics and interface interpretation

Part 2 | A dark cavity has several mechanisms

Read retinal thickness and layer continuity before labelling oedema.

Schematic OCT sections. A dark space is not automatically treatable oedema.

DME: thickened



Oedematous fluid

MacTel: thin cavities



Degenerative cavitation

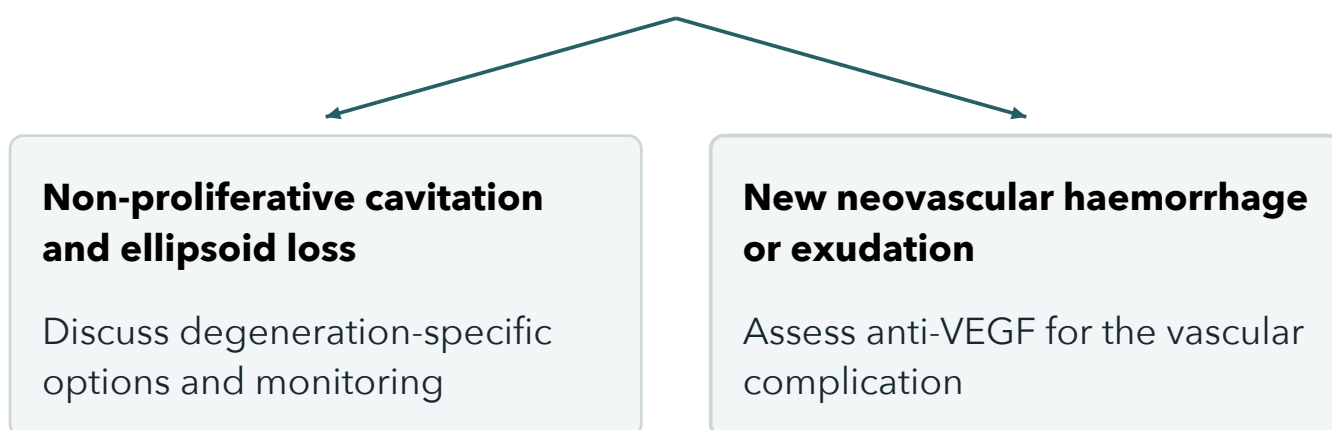
Original comparison of oedematous cysts and non-swollen degenerative cavities.

A scan is a set of anatomical observations, not a treatment command. Describe where the space lies, which layers remain continuous, whether the retina is swollen and what the surface is doing. Then test the interpretation against function and angiography.

MacTel: degeneration versus neovascular complication

Disease stage changes the purpose of treatment.

Which MacTel phenotype is present?



Avoid the obsolete blanket claim that no MacTel treatment exists. Separate a treatment intended to slow degeneration from treatment of a secondary neovascular lesion, and explain which outcome is being pursued.

Traction: interpret the surface and the direction

Attachment becomes clinically important when it distorts structure or function.

Finding	Implication
Vitreous attachment	May be present without retinal distortion
Vitreomacular traction	Attachment with a tractional structural effect
Epiretinal membrane	Tangential distortion and metamorphopsia
Mixed interface disease	More than one tractional component

An incidental membrane is not a universal surgical indication. Document meaningful functional symptoms, other macular pathology and the structural prognosis before proposing a peel. A normal central acuity does not measure all troublesome distortion.

Pseudohole lamellar defect and full-thickness hole

Contour alone does not establish a full-thickness tissue defect.

Finding	Implication
Pseudohole	Steep contour, often membrane-related; no full-thickness loss
Lamellar defect	Partial-thickness tissue abnormality
Full-thickness hole	Complete foveal retinal defect
Myopic schisis	Retinal splitting with additional geometric forces

The value of the classification is the different mechanism and management discussion it supports. Do not convert every steep foveal contour into a macular-hole operation or every split into fluid needing an injection.

Myopic traction: geometry matters

A staphyloma can interact with vitreous and membrane traction.

Finding	Implication
Schisis	Intraretinal separation
Foveal detachment	A different plane from schisis
Macular hole	Assess whether retinal detachment also exists
Outer retina	Prognostic information beyond anatomical repair

State both the internal traction and the external globe geometry. An operation that relieves one force may not eliminate the other. Anatomical improvement and restored visual function should be discussed as related but distinct goals.